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This instruction implements Air Force Instruction (AFI) 44-172, *Mental Health,* and sections of *AFSOC Concept of Employment for Wing A-Staff – A3M (Medical Operations Division)* applicable to Integrated Operational Support Mental Health (IOS MH). This publication applies to all active duty Air Force Special Operations Command (AFSOC) operational medical personnel and Integrated Operational Support (IOS) program personnel. This instruction applies to AFSOC gained Air Force Reserve Command (AFRC) and Air National Guard (ANG) units, where "MDG" or "MTF" may be replaced with the applicable level of medical unit. This publication does not apply to the United States Space Force. This publication requires the collection and/or maintenance of information protected by 5 USC Section 552a, *Records Maintained on Individuals*, subsequently referred to as the "Privacy Act of 1974", as amended. Forms affected by the PA have an appropriate PA statement. System of Records Notices F044 AF SG D, *Automated Medical/Dental Record System*, F044 F SG E, *Electronic Medical Records System* are available at: **https://pclt.defense.gov/DIRECTORATES/Privacy-and-Civil-Liberties-Directorate/Privacy/SORNsIndex/DoD-Component-Notices/Air-Force-Article-List/**. Additionally, EDHA 07, *Military Health Information System*, is available at: **https://pclt.defense.gov/DIRECTORATES/Privacy-and-Civil-Liberties-Directorate/Privacy/SORNsIndex/DoD-Component-Notices/DHA-Article-List/**. Ensure all records generated as a result of processes prescribed in this publication adhere to Air Force Instruction (AFI) 33-322, *Records Management and Information Governance Program*, and are disposed in accordance with the Air Force Records Disposition Schedule, which is located in the Air Force Records Information Management System. Refer recommended changes and questions about this publication to the Office of Primary Responsibility (OPR) using the Department of the

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Chapter 1

AFSOC IOS MH SERVICES

1.1. Introduction and Overview. DAFMAN 10-211 provides guidance and procedures for DAF IOS programs. AFI 44-172, *Mental Health*, provides similar guidance for DAF mental health readiness and procedures applicable to AFSOC IOS MH. This publication implements AFSOC procedures for IOS MH services. The intent of this publication is to build upon DAFMAN 10-211 and AFI 44-172 to provide further clarity for the day-to-day operations and unique programs for AFSOC's IOS MH services.

1.2. Terminology. "IOS MH" is the term used in the DAF to describe mental health care performed outside of the Medical Treatment Facility (MTF). The terminology builds on the concept of "Limited Scope Counseling" that was originally described in AFMAN 48-149, *Flight and Operational Medicine Program (FOMP)*, but the concept has been re-defined and expanded with lessons learned since Limited Scope Counseling was first envisioned. Further, the "Limited Scope Counseling" term was problematic due to differing interpretations regarding its scope of care, and ultimately the term was changed to "Integrated Operational Support Mental Health (IOS MH)" in DAFMAN 10-211. Finally, the term "integrated" is now used by Air Force Medical Command (AFMEDCOM) to help clearly delineate DAF-funded medical operations, rather than the term "embedded" which is used by other Services and the Defense Health Agency (DHA) and carries various meanings.

1.2.1. IOS MH programs provide Operational Clinical Services (OCS); i.e., clinical care that is delivered inside a unit setting, outside of the MTF/Mental Health Flight, and is documented in the Electronic Health Record (EHR; i.e., Military Health System's GENESIS).

1.2.2. IOS MH is distinguished from other important, but non-clinical efforts and services, such as operational psychology, psychoeducation, commander consultation regarding routine unit issues, unit walkabouts, and other non-clinical treatment services.

1.3. IOS MH Personnel. IOS MH personnel include mental health providers and technicians. IOS MH providers are licensed mental health clinicians credentialed and privileged by their host MTF. IOS MH providers include Active Duty, General Schedule civilians, and Contractors. IOS MH technicians assist their IOS MH providers and operate under their providers' license or within their scope of practice if separately privileged (e.g., Certified Alcohol and Drug Counselor).

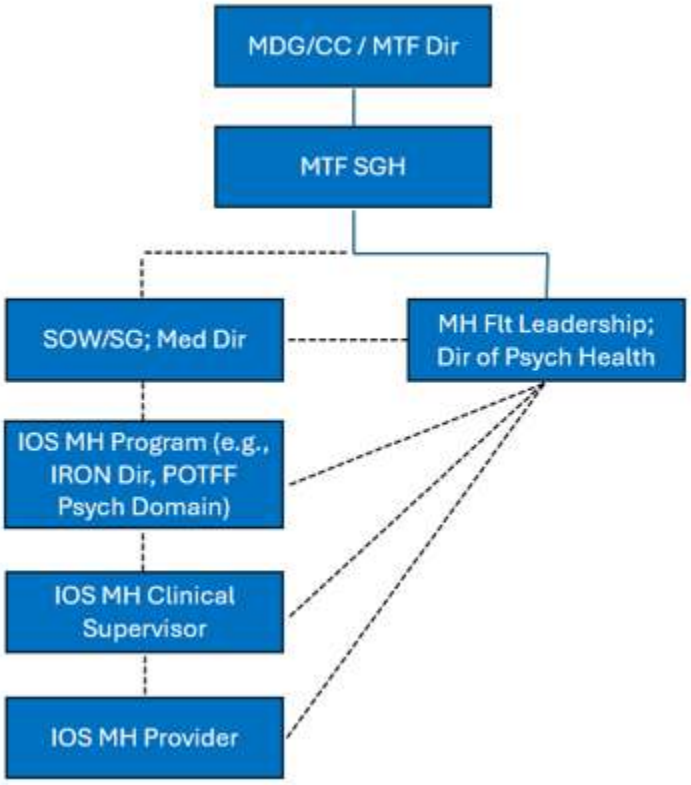
1.3.1. Organizationally, IOS MH providers and technicians can belong to different IOS MH programs with their own respective administrative control, including (but not limited to): United States Special Operations Command's (USSOCOM) Preservation of the Force and Family (POTFF), AFSOC's Integrated Resilience Optimization Network (IRON), DAF's True North+, and Active Duty/Guard/Reserve mental health personnel. These programs and organizations have their own policy guidance that will be followed by their respective personnel. If conflicting guidance is found between the various IOS MH programs' respective policies and this guidance, AFSOC/SGPY and/or AFSOC/A1Z may serve as OPRs to deconflict and provide updated guidance.

1.3.2. DAFMAN 10-211, paragraph 5.2.1., notes that "Licensed Marriage and Family Therapists are not authorized in IOS programs." This specific prohibition was intended to ensure that marital and family therapy do not occur in the IOS setting. The prohibition against use of Licensed Marriage and Family Therapists (LMFT) as a provider labor category does not apply to the POTFF program within AFSOC because LMFTs are authorized by the DHA, currently credentialed and privileged by AFSOC MTFs, POTFF is a USSOCOM IOS program (not DAF IOS), and the IOS MH care provided by LMFTs do not include marital or family therapy in the IOS setting (see **paragraph 2.3.11**.). **Note:** this one DHA-related exception to a provider labor category does not negate the remaining applicability of DAFMAN 10-211 to the POTFF program.

1.4. MTF and SOW/A3M Oversight of IOS MH care. IAW DoDI 6025.13, *Medical Quality Assurance and Clinical Quality Management in the Military Health System,* and DAFMAN 10-211, ultimately all medical care is under MTF oversight, including IOS MH care. Clinical decision-making for IOS MH care is subject to the same medical standards as care within the MTF. While the MTF may not "own" the IOS MH medic, the MTF functionally "owns" the medical care provided by IOS MH personnel. Further, IAW *AFSOC Concept of Employment for Wing A-Staff – A3M (Medical Operations Division)*, the wing's and MTF's primary OPR for IOS personnel and IOS care is the wing A3M. The Medical Operations Function within A3M, led by the Special Operations Wing Surgeon (SOW/SG), is responsible for the functional oversight and coordination authority of all integrated operational medical personnel – to include IOS MH – and ensures that these medics meet in-garrison clinical requirements, comply with host medical treatment by-laws, and serves as the primary liaison with the MTF for IOS care. IAW the A3M CONEMP, senior medical officers (e.g., Medical Director) who advise group or unit commanders that fall outside of a traditional wing structure execute the same duties as the SOW/SG as they pertain to the unit's respective mission.

1.4.1. Typically, the "functional chain of command" for IOS MH care flows from the MDG/CC / MTF Director, to the MTF SGH, to the SOW/SG (or equivalent Medical Director), with active functional leadership inputs from the MTF/Mental Health Flight and the IOS MH programs. While the SOW/SG is the wing's functional oversight and coordination authority of IOS MH, it is not expected that the SOW/SG will have the expertise and experience for the day-to-day IOS MH operations and will thus actively rely upon the functional expertise of wing assigned mental health assets (e.g., wing AD psychologist, IOS MH program) and the MTF/Mental Health Flight. The SOW/SG ensures functional partnerships exist between the wing's IOS MH programs and the MTF/Mental Health Flight. **Note:** functional oversight is not administrative oversight.

Figure 1.1. "Typical" IOS MH Functional Chain of Command.



1.4.2. Typically, the MTF's OPR for functional leadership over specialty mental health care flows from the MTF SGH to the AD Mental Health Flight Leadership (e.g., Mental Health Flight Commander, Guard and Reserve Director of Psychological Health roles). These roles are the SMEs for specialty mental health, including currency and competency in clinical treatment and processes, policy (including DAF, AFMEDCOM, and DHA), and military mental health medical standards. This mental health expertise and functional leadership is necessary for both the MTF's and SOW/SG's oversight of IOS MH care, and the MTF/Mental Health Flight (or equivalent) should be actively involved as a SME to provide guidance to MH IOS clinical decision-making and actively partner with the SOW/SG for effective IOS MH oversight. SME and liaison roles with the SOW/SG may include (but are not limited to): oversight and quality assurance of training MH IOS personnel, oversight of peer review and mental health clinical documentation, and clinical consultation.

1.4.3. AFSOC has MH IOS programs in a variety of diverse locations not traditionally found across the Air Force Medical Service. AFSOC MH IOS programs may reside on installations without an A3M, be geographically separated from their host MTF/Mental Health Flight, assigned to an MTF from another MAJCOM or service component, or even be located on a non-Air Force installation. In these scenarios (but not limited to), the functional leadership for the IOS MH program may reside at the IOS location and require an active and collaborative working partnership with the geographically separated SOW/SG (or equivalent Medical Director) and/or host MTF/Mental Health Flight. Regardless of geographical distance or assigned MTF/Mental Health Flight, AFSOC IOS MH programs are subject to the same policies and procedures and will have established working relationships with the host

MTF/Mental Health Flight to ensure compliance and oversight (e.g., MOUs). AFSOC/SGPY and/or AFSOC/A1Z will serve as OPRs to ensure any such arrangements are compliant and inspectable with existing guidance.

1.5. IOS MH, A3M, and MTF Partnership. It is essential for the SOW/SG, MTF/Mental Health Flight, and the IOS MH programs to have an active and collaborative working relationship to ensure quality care is provided and to resolve challenges as they arise. The MTF should foster a close working relationship with the SOW/SG and IOS MH personnel, include IOS MH personnel in MTF trainings and meetings (e.g., ProStaff, Mental Health Multi-Clinic Case Conference Meeting), be knowledgeable of IOS MH programs' policies and capabilities, and support IOS MH programs to the same level as MTF programs. The SOW/SG and IOS MH personnel will likewise actively collaborate with the MTF/Mental Health Flight, to include (but not limited to) attend MTF trainings and meetings, be knowledgeable of DAF, AFMEDCOM, and DHA policies and capabilities, and ensure that their clinical care meets MTF standards.

1.6. No Limit to Number of IOS MH Appointments. IOS MH services are not inherently limited in number of appointments or length of time receiving care, though the treatment will adhere to the same medical standards as care provided by a MTF provider (e.g., including considerations of fitness and suitability for duty, weapons bearing, flight status, deployability, retention standards, etc.).

Chapter 2

APPROPRIATENESS OF IOS MH SERVICES

2.1. Role of Clinical Judgment for IOS MH. Some conditions or issues are inherently inappropriate for IOS MH, but for others it may be unclear and require a clinical judgment call to determine whether IOS MH care is appropriate. It is incumbent on IOS MH functional chain of command to actively partner regarding clinical decision-making. Such collaboration could involve as needed or just in time consultation, but it is strongly encouraged for there to be a recurring sync (e.g., a recurring installation IOS MH and MTF/Mental Health Flight meeting).

2.2. Guidelines for Determining Appropriateness of IOS MH. It is not feasible to determine ahead of time all potential conditions and clinical severity levels regarding what is appropriate for IOS MH vs. specialty care provided in the Mental Health Flight, but the following should be considered when making a clinical judgment call:

2.2.1. Low Acuity. This means that the IOS MH patient has a less severe medical condition, requiring less clinical intervention and less intensive monitoring, as compared to a "high acuity" patient. IOS MH patients do not possess life-threatening or significant duty-impacting conditions that are the primary focus of IOS MH treatment. IOS MH patients do not require frequent appointments to function in their duties. IOS MH, by definition, is less intensive than the higher level of care provided in the Mental Health Flight.

2.2.2. Low Complexity. This means that the IOS MH patient has relatively stable and straightforward condition(s) or issue(s), meaning they have few or no complicating factors, require relatively minimal decision-making, and generally have uncomplicated treatment plans. Typically, IOS MH care involves at most a few relatively minor conditions or issues, or a single stable chronic condition or issue, with minimal management needs. The IOS MH patient is likely to be well-managed with routine IOS MH care and follow-up.

2.2.3. Low Risk. This means that the IOS MH patient is identified as low risk for danger for suicide, low risk for danger to self (e.g., non-suicidal self-injurious behavior), or low risk to others, and is not on the Mental Health Flight's At Risk Tracking List (ARTL). For danger to self or danger to others, there is no current DAF, AFMEDCOM, or DHA identified means to measure this type of non-suicidal risk, which requires the IOS MH provider to use clinical judgment when determining the appropriateness of IOS MH vs. specialty mental health care in the Mental Health Flight. For danger for suicide risk, IOS MH may be utilized provided patients are identified as "low acute risk" or "low chronic risk" as defined by the current version of the *VA/DoD Clinical Practice Guidelines for the Assessment and Management of Patients at Risk for Suicide* and DHA-AI 6025.06, *Suicide Risk Care Pathway for Adult Patients in the Defense Health Agency*. In addition to clinical judgment, a Columbia-Suicide Severity Rating Scale will be utilized to determine suicide risk level and will be given to the IOS MH patient at the first appointment and thereafter as clinically indicated and/or as required by DHA guidance. Patients determined to be at intermediate or higher risk for suicide (acute or chronic) will be referred to the Mental Health Flight for treatment and clinical management. **Note:** the IOS MH provider may serve as the referred provider to treat the higher risk suicidal patient at the Mental Health Flight (see **paragraph 2.4**.).

2.2.4. When treating IOS MH patients who are identified to have a low or no risk, it is incumbent on the IOS MH provider to provide clear justification and demonstration of risk in the clinical documentation, in line with existing DAF, DHA, and AFMEDCOM guidance (e.g., management of suicidal patients). It is strongly advised that the IOS MH provider consults initially and as needed with their IOS MH clinical supervisor and/or the MTF/Mental Health Flight's leadership when treating IOS MH patients with any risk-related needs and then document the clinical rationale in the EHR.

2.2.5. Active IOS MH patients who are assessed to be higher than low risk will be transferred for care at the MTF/Mental Health Flight and be considered for inclusion on the ARTL (see **paragraph 2.4**.). When a Mental Health Flight patient reverts to low or no risk, the patient may be considered for IOS MH care again in the IOS location, provided the patient is not currently on the ARTL, the patient is stable (consider acuity and complexity), and a clinical decision is made regarding appropriateness of IOS MH care.

2.3. Mental Health Services Prohibited in an IOS MH Setting. Complementing DAFMAN 10-211 and AFI 44-172, the following services will not be provided outside of the MTF. These services require the resources and personnel of an MTF to provide safe and effective patient care at a higher level of care than what is appropriate for IOS MH.

2.3.1. Treatment of patients at either acute or chronic intermediate or higher risk for suicide or danger to self (e.g., non-suicidal self-injurious behavior)

2.3.2. Treatment of patients at elevated risk for danger to others (requires a clinical judgment call)

2.3.3. Treatment of current Mental Health Flight ARTL patients

2.3.3.1. IAW DAFMAN 10-211, IOS MH providers will deliver MH care for ARTL patients at the MTF/Mental Health Flight. It is understood that in some geographically isolated locations in AFSOC, this requirement necessitates patients to travel a considerable distance for care, but by definition ARTL patients require a higher level of care than what can be provided in the IOS location. The MTF/Mental Health Flight (or service equivalent) owns the ARTL process to manage patients with higher levels of risk; thus, IOS MH programs follow the host MTF/Mental Health Flight processes. In this scenario, the IOS MH program and host MTF/Mental Health Flight actively partner to mutually support one another to manage patient risk, meet patient care and commander needs, and document the steps and responsibilities necessary to ensure compliance (e.g., Memorandum of Agreement or Memorandum of Understanding). As needed, AFSOC/SGPY and/or AFSOC/A1Z will review, verify compliance, and adjudicate any exception to policy requests.

2.3.3.2. IAW DAFMAN 10-211, all IOS MH personnel who are treating a MH ARTL patient will attend the MTF/Mental Health Flight At Risk Tracker (ART) meeting.

2.3.4. Treatment of Alcohol Drug Abuse Prevention and Treatment patients (i.e., IOS MH does not include substance use related treatment; any such needs are referred to the Mental Health Flight for care)

2.3.5. Treatment of Family Advocacy Program patients (i.e., IOS MH does not include family maltreatment related treatment; any such needs are referred to the Mental Health Flight for care)

2.3.6. Treatment of complex cases requiring multi-disciplinary coordination. IOS MH-level care may be appropriate for patients with multidisciplinary or multi-clinic specialty mental health related needs (e.g., IOS MH, plus substance abuse treatment, plus psychiatry), provided that the IOS MH provider participates in the Mental Health ART meeting to coordinate care and that the MH IOS care is an adjunct to the patient's higher level care multi-clinic treatment needs. See paragraph **2.4 and paragraph 2.4.1** regarding the IOS MH provider assuming the responsibilities of a specialty mental health care patient.

2.3.7. Sanity Boards or Forensic Evaluations

2.3.8. Command-Directed Evaluations, including both routine and emergent

2.3.9. Medical Evaluation Boards (MEB). The appointment(s) necessary to evaluate a patient for a MH-related MEB must be done in the MTF but a patient who is otherwise undergoing or awaiting the results of the MEB process may be treated with IOS MH.

2.3.10. Treatment of patients with impaired reality testing

2.3.11. Family Therapy or Marital Therapy (**Note:** dependents can only be seen for documented mental health care inside the MTF/Mental Health Flight. This kind of medical treatment is distinguished from non-documented, non-medical family or marital related concerns that may be appropriate for discussion (not treatment) in the IOS MH setting.)

2.4. IOS MH Personnel Providing Care Inside the MTF/Mental Health Flight. IOS MH personnel are intended to provide IOS MH-level care in the IOS setting; however, IOS MH personnel may also provide specialty mental health services (i.e., a higher level of care than IOS MH) inside of the MTF/Mental Health Flight IAW their credentials and privileges to maintain continuity of care for their unit members, IAW DAFMAN 10-211. For example, an IOS MH provider may treat a member of their assigned unit referred for care in the Mental Health Flight – to include the scenario when their unit member is on the ARTL or write the narrative summary for their unit member referred for an MEB. When an IOS MH patient is transferred to the MTF/Mental Health Flight, IOS MH care ceases and is replaced by specialty mental health care. IOS MH providers should be considered as the first option to continue care of the patient when their unit members are referred for care to the Mental Health Flight; however, this will be balanced with the reality that IOS MH providers are intended for IOS MH care and should also only have a portion of their caseload be specialty mental health patients. Further, it may not be feasible or clinically appropriate for the IOS MH provider to always be the Primary Mental Health Provider for a patient referred to the MTF/Mental Health Flight from their unit. In these and similar cases, a collaborative consultation between the IOS MH provider, IOS MH clinical supervisor, and the MTF/Mental Health Flight leadership should occur to determine who the treating provider will be. Ultimately, the MTF/Mental Health Flight leadership retains functional authority regarding the specialty mental health care of providers inside the Mental Health Flight and ensures that care is accomplished by personnel who are fully trained, credentialed and privileged.

2.4.1. It is permissible for an IOS MH provider to serve as the Primary Mental Health Provider when treating a patient inside the MTF/Mental Health Flight for a patient requiring multidisciplinary or multi-clinic mental health related needs.

2.4.2. The role of the Primary Mental Health Provider is intended to serve as the lead mental health provider to ensure coordination of care with other providers (including PCMs) and communicate any duty or mobility restrictions with the patient's Commander. When in this capacity, the IOS MH provider will attend the ART and fully participate with other providers to coordinate care for the patient.

Chapter 3

IOS MH PATIENT AND PERSONNEL SAFETY

3.1. IOS MH Safety Measures. IOS MH is intended for non-emergent conditions and issues (low acuity, low complexity, and low risk); nevertheless, per DAFMAN 10-211, IOS MH programs will be provided in locations that have safety measures for emergency situations. The unit that owns the IOS location will develop an SOP or equivalent to define this requirement. The SOW/SG will validate the safety measures SOP from a functional perspective.

3.2. Operating Hours and Availability of Assistance. IOS MH will only occur within unit operational hours and only be provided if there are other individuals physically present in the unit and available to assist should an emergent situation develop. IOS MH personnel normally defer any after-hours emergent issues to the MTF on-call mental health provider or address any routine issues the following duty day. **Note:** this provision assumes "normal" duty hours (not an IOS MH provider engaged in approved 24/7 operations), nor does it apply if IOS MH personnel are assisting with a Disaster Mental Health team response and are responding in that capacity.

3.2.1. For IOS MH programs with 24/7 operations or that are geographically separated from their host MTF/Mental Health Flight, the IOS MH program and MTF/Mental Health Flight should establish a written plan to address support and notification requirements when the host MTF/Mental Health Flight is unavailable to the IOS MH program. Examples include a plan when the MTF is closed but the IOS MH program is not, or when an IOS MH patient unexpectedly needs off base emergent care outside of normal MTF/Mental Health Flight duty hours.

3.2.2. The written plan to address support and notification requirements should also address the timing and notification process to facilitate the MTF/Mental Health Flight's ARTL and patient tracking responsibilities.

3.3. Warm Hand Off of Care. When referring an IOS MH patient to the Mental Health Flight (including the scenario when the IOS MH provider will continue to treat the patient in the Mental Health Flight) or if facilitating a referral to a higher level of care, IOS MH personnel will ensure an explicit warm hand off is accomplished with the Mental Health Flight. IOS MH personnel will follow the Mental Health Flight's processes for warm handoffs. The intention is to ensure awareness and collaboration to provide for the patient's needs, and that all parties understand the clinical situation, plan, and the roles that the IOS MH provider and Mental Health Flight will assume for the patient (e.g., who "owns" the patient as the primary mental health provider?). When referring patients at elevated risk (e.g., when the patient requires entry on the ARTL), the warm handoff will also define who will implement ARTL procedures and notifications (e.g., command notification for entry on the ARTL, initiation of a medical profile, entry on the ARTL roster, etc.).

3.3.1. IOS MH personnel will not transport patients, including patients who require or are suspected of needing higher level of care.

3.3.2. IOS MH personnel may assist in coordinating transportation for patients to emergency services following the same process as the local MTF/Mental Health Flight.

Chapter 4

GUIDELINES FOR IOS MH

4.1. Credentialing and Privileging. IAW DAFMAN 10-211, credentialing and privileging for IOS MH personnel will be IAW DoDI 6025.13 and DHA-PM 6025.13, *Clinical Quality Management in the Military Health System Volume 4: Credentialing and Privileging*. IOS MH personnel must meet and maintain all their credentialing and privileging requirements IAW their employment and readiness requirements. Further, IOS MH personnel are responsible for maintaining their license and/or board certification with sufficient continuing education in addition to meeting credentialing and privileging requirements as applicable. Requirements include (but not limited to) adhering to MTF Medical Staff Bylaws, fulfilling MTF training requirements, completing initial Focused Professional Practice Evaluation, participating in Ongoing Professional Practice Evaluations, and ongoing maintenance of clinical privileges to include meeting the current DHA standard of accumulating sufficient peer reviewable clinical hours.

4.1.1. IOS MH personnel function as MTF affiliate providers and are subject to DHA requirements to maintain their clinical privileges. This includes practicing the minimum number of hours of peer reviewable care to maintain privileges with the MTF.

4.1.2. IAW DHA policy regarding MTF Privileging Authority and Exception to Policy authority, the MTF may define the type of clinical care necessary to meet the annual requirement.

4.2. Eligibility for IOS MH Care. IAW DAFMAN 10-211, IOS MH is intended only for uniformed members. While dependents may attend a service member's IOS MH appointment inside the unit (e.g., to provide collateral information or assist the provider treating the service member patient), the intention is for IOS MH to be reserved for uniformed members as the primary customer. Dependents may be seen for documented mental health services in the MTF if services are available and approved by the MTF. Dependents may also be seen for non-clinical services provided by IOS MH personnel, as available and permitted by the IOS MH program.

4.3. Productivity Guidelines. Productivity guidelines for IOS MH personnel differ from DHA personnel who work inside the MTF. IOS MH providers and technicians are dedicated unit assets, not DHA employees, and they are not subject to DHA's productivity requirements, nor can they be utilized by the MTF to supplement MTF manning (DAFMAN 10-211 paragraph 2.8.4.). IOS MH personnel are subject to their own IOS MH program's requirements for productivity considerations.

4.4. HIPAA and IOS MH. The content of IOS MH sessions, as a form of medical care, is protected from unauthorized disclosure and subject to the same confidentiality and disclosure regulations as care within the MTF. IOS MH providers are licensed professionals who must ensure there is no undue command influence in their clinical decision-making, including any duty restrictions necessitated by a medical or mental health condition. If IOS MH personnel feel pressure or undue influence from command regarding clinical decision-making, duty restrictions, disclosure of health information, etc., they will follow their functional chain of command to resolve the situation.

4.5. Mandatory Reporting. Regardless of whether care is rendered inside or outside of the MTF, IOS MH personnel are required to follow the same mandatory reporting requirements as providers inside the MTF, IAW DoDI 6490.08, *Command Notification Requirements to Dispel Stigmas in Providing Mental Health Care to Service Members*. Situations include (but are not limited to): harm to self, harm to others, harm to mission, special duty status (e.g., flying status, PRP, PSP, etc.), inpatient care, acute medical condition(s) interfering with duty, substance abuse treatment, command directed mental health evaluations, and "other special circumstances" (DoDI 6490.08). When consulting with unit leadership, IOS MH personnel may only disclose the minimum necessary information to a commander with G-series orders or the commander's written designee (DoDI 6490.08) and will adhere to their MTF's policies for accounting for disclosures to Command Authorities.

4.6. Informed Consent. IOS MH will require informed consent and will follow existing DHA and MTF requirements using the same informed consent document utilized in the MTF/Mental Health Flight for IOS MH care, though additional IOS MH program specific information may be added to the document. The IOS MH patient will be provided with an informed consent document prior to beginning the first session, and the IOS MH provider will provide an explanation of informed consent and limits of confidentiality associated with IOS MH. The informed consent document will be uploaded into the EHR.

4.7. Peer Review. IOS MH requires peer review IAW DHA and MTF Bylaws, procedures, external accreditation organization standards. Peer review data must be submitted to and reviewed by the MTF.

4.7.1. IAW DHA and MTF Medical Staff Bylaws, IOS MH personnel may be asked to participate in Standard of Care reviews or Quality of Assurance Investigations, including requests from other installations.

4.7.2. Should IOS MH personnel believe that there is a significant conflict of interest preventing them from participating in these activities, they will consult with their IOS MH clinical supervisor and/or their functional chain of command for resolution.

4.8. Documentation and Coding. IAW DoDI 6490.13, *Comprehensive Policy on Traumatic Brain Injury-Related Neurocognitive Assessments by the Military Services*, IOS MH requires clinical documentation in the EHR following the same DHA and AFMEDCOM standards and coding requirements that are applicable to care done inside the MTF. This includes documentation in the EHR regarding communication with patients about their IOS MH medical appointments, clinical consultations, relevant treatment updates, receipt and other coordination of care activities. Using clinical judgment, IOS MH personnel may wish to document "non-clinical care", such as receipt of non-IOS MH care that has the potential for future diagnosis or treatment, or validation of medical readiness (e.g., assisting the MDG to validate medical readiness for a member receiving non-IOS MH services when facing an upcoming deployment).

4.8.1. IOS MH programs may utilize their own additional documentation requirements or systems that are beyond the scope of this instruction and are subject to their respective program's oversight and directives

4.8.2. Any such additional requirements or systems do not replace the requirement to document IOS MH in the EHR.

4.9. Behavioral Health Data Portal (BHDP). IOS MH will use the BHDP for all patients, just like care done inside the Mental Health Flight, to meet the same clinical outcome measures and program requirements for DAF, AFMEDCOM, DHA, and USSOCOM. However, in some locations across AFSOC, geographical and/or technological safeguards exist that will limit or even prohibit BHDP use (e.g., IOS MH inside a Secure Compartmented Information Facility, poor or no cell phone coverage for the remote BHDP option, etc.). When significant limitations exist, IOS MH will collaborate with AFSOC/SGPY and/or AFSOC/A1Z to overcome the challenges or receive a waiver to discontinue BHDP use if appropriate. Whenever BHDP use is not possible, IOS MH personnel will utilize hard copy screening measures to obtain relevant clinical data as indicated.

4.10. Physical Space Requirements. IOS MH will be conducted in a private area provided by the unit where the following criteria are met: (a) minimal sound transmission (to ensure privacy), (b) appropriate lighting, and (c) appropriate seating to ensure privacy. Typically, IOS MH is conducted in a private room or office; however, some AFSOC locations may require alternative arrangements. To the best of their ability, IOS MH personnel will be mindful of their surroundings to ensure appropriate protection of PHI when providing IOS MH and documenting care. The physical spaces where IOS MH care is provided will also adhere to the host MTF's TJC accreditation requirements for convenience level care.

4.10.1. IOS MH is intended to be provided in unit settings (the "IOS location" IAW DAFMAN 10-211, paragraph 5.2.3.1.).

4.10.2. IOS MH providers will not provide care in service member's homes, cars, or in a public location such as a coffee shop or restaurant.

4.11. Boundaries. Appropriate relational boundaries with individuals in the unit will be maintained IAW DoD, DHA, AFMEDCOM, and state licensure requirements. There will be some instances where professional boundaries will dictate the necessity of referring unit members to a different IOS MH provider or technician, or to the MTF/Mental Health Flight for treatment. IOS MH personnel will rely on their training, experience, and consultation to identify these circumstances (e.g., undue familiarity with the member, or when providing treatment would be disruptive to workplace relationships, etc.). When these situations occur, IOS MH personnel should consult with their respective IOS MH clinical supervisor and/or with the MTF/Mental Health Flight leadership for appropriateness of the referral.

4.12. Affiliate Provider. As affiliate providers, IOS MH personnel will be onboarded and maintain an active working relationship with their host MTF and Mental Health Flight. This includes attending the MTF Newcomer's Orientation (or equivalent), receiving an orientation to the Mental Health Flight, attending ongoing relevant Mental Health Flight meetings and training, fulfilling the training requirements of a new MTF mental health provider, and out-processing the MTF when transitioning out of an IOS MH role.

4.13. Geographically Separated Locations. For those AFSOC locations with IOS MH programs geographically separated from the host MTF/Mental Health Flight, the IOS MH functional leadership (which may be located at the IOS location) will collaborate with the host MTF/Mental Health Flight to ensure orientation and onboarding are accomplished to the satisfaction of the host MTF/Mental Health Flight.

Chapter 5

IOS MH CLINICAL SUPERVISION

5.1. Clinical Supervisory Roles will Vary. AFSOC has diverse needs and has multiple IOS MH organizational models; thus, IOS MH clinical and administrative supervisory roles at one base or GSU may not resemble roles at another location (e.g., AD vs. Reserve SOWs; POTFF, True North+, and IRON organizational models; AD Wing Psychologist vs. GS civilian Psychological Domain Lead). IOS MH programs such as POTFF and True North+ have established designated administrative supervisory roles and relationships for clinical oversight of IOS MH, such as the POTFF Psychological Domain Lead. These already established roles provide valuable and necessary oversight for IOS MH and meet the intent of DAFMAN 10-211 paragraph 4.2.2., 4.2.3., and 4.2.4.

5.2. Identification of IOS MH Clinical Supervision. Within AFSOC, the SOW/SG (or equivalent) will work with the IOS MH program to identify in writing clinical supervisory roles and relationships to ensure unambiguous clinical oversight of IOS MH care. An example (but not limited to) is an MFR signed by the IOS MH program and SOW/SG listing IOS MH personnel and their clinical supervision. Potential clinical supervisors of IOS MH care could include (but are not limited to) POTFF/IRON Psychological Domain Leads or a unit assigned AD mental health provider (e.g., wing operational psychologist).

5.2.1. If the clinical supervisor is routinely unavailable to provide adequate clinical supervisory duties, a new clinical supervisor will be identified. It is recommended to follow the functional chain of command to identify a new clinical supervisor, and if necessary AFSOC/SGPY and/or AFSOC/A1Z may assist in this process.

5.2.2. When an IOS MH program does not inherently have an identified clinical supervisor (e.g., low manning, or if a Reserve SOW has only one IOS MH provider in total for their program), the IOS MH program will work with their SOW/SG or medical director (if applicable), local MTF, the Mental Health Flight, and/or any other local IOS MH program to identify a clinical supervisor. If needed, AFSOC/SGPY and AFSOC/A1Z will collaborate to assist with identifying clear supervisor roles and relationships.

5.3. IOS MH Clinical Supervisory Duties include:

5.3.1. Provides clinical oversight for IOS MH, to include an increased clinical oversight for IOS MH care involving patients who are at elevated risk for danger to self, others, or to the mission, and ensures that such care follows established treatment guidance and protocols (e.g., ensures that patients are appropriate for care in an IOS MH setting vs. care in the Mental Health Flight).

5.3.2. Assists IOS MH program leadership and the SOW/SG to ensure that the IOS MH program maintains a close working relationship with the MTF and Mental Health Flight.

5.3.3. Liaises with the IOS MH program, line leadership, SOW/SG, MTF/Mental Health Flight, and other IOS MH or helping agencies on the installation to ensure quality of care and to help resolve challenges as they arise.

5.3.4. Ensures that IOS MH personnel follow the guidance of the SOW/SG and MTF/Mental Health Flight, and are oriented to the MTF and Mental Health Flight to the same standard as MTF providers. Also ensures that IOS MH personnel appropriately out-process the MTF/Mental Health Flight when departing the role.

5.3.5. Ensures compliance with guidance and appropriate clinical practice, including credentialing and privileging, completion of the requirements to maintain credentials, peer review, and updates on DoD, DHA, AFMEDCOM, and MTF clinical developments, policies, and training requirements.

5.3.6. Ensures participation of IOS MH personnel in clinic, MTF meetings, and trainings.

MICHAEL B. BROUGH, Colonel, USAF, MC, CFS
Command Surgeon, Air Force Special Operations Command

Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

References

5 USC § 552a*, Records maintained on individuals* (Privacy Act of 1974)

AFI 33-322, *Records Management and Information Governance Program*, 23 March 2020

AFI 44-172, *Mental Health*, 13 November 2015

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AFSOC Concept of Employment for Wing A-Staff – A3M (Medical Operations Division), 27 October 2025

DAFMAN 10-211, *Integrated Operational Support (IOS)*, 21 January 2026

DHA-AI 6025.06, *Suicide Risk Care Pathway for Adult Patients in the Defense Health Agency*, 9 August 2022

DHA-PM 6025.13, *Clinical Quality Management in the Military Health System Volume 4: Credentialing and Privileging,* 26 July 2023

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DoDI 6490.08, *Command Notification Requirements to Dispel Stigmas in Providing Mental Health Care to Service Members*, 6 September 2023

DoDI 6490.13, *Comprehensive Policy on Traumatic Brain Injury-Related Neurocognitive Assessments by the Military Services*, 11 September 2015

VA/DoD Clinical Practice Guideline, *Assessment and Management of Patients at Risk for Suicide*, Version 3.0, April 2024

Prescribed Forms

None

Adopted Forms

DAF Form 847, *Recommendation for Change of Product*

Abbreviations and Acronyms

A3M—Medical Operations Division

AFMEDCOM—Air Force Medical Command

AFSOC—Air Force Special Operations Command

ART—At Risk Tracking

ARTL—At Risk Tracking List

BHDP—Behavioral Health Data Portal

DAF—Department of the Air Force

DHA—Defense Health Agency

EHR—Electronic Health Record

IOS—Integrated Operational Support

IOS MH—Integrated Operational Support Mental Health

IRON—Integrated Resilience Optimization Network

MEB—Medical Evaluation Board

MTF—Medical Treatment Facility

POTFF—Preservation of the Force and Family

SG—Surgeon General

SGH—Chief of the Medical Staff

USSOCOM—United States Special Operations Command

Office Symbols

AFSOC/SGPY—AFSOC Command Psychologist

AFSOC/A1Z—AFSOC Integrated Resilience

Wg/SG—A3M Wing Surgeon

Terms

Integrated Operational Support (IOS)—Line funded programs with at least one medical authorization. (DAFMAN 10-211)

Integrated Operational Support Mental Health (IOS MH)—the term used in the DAF to describe mental health care performed outside of the Medical Treatment Facility. (DAFMAN 10-211)

Limited Scope Counseling—an obsolete term used to describe short-term counseling that is limited in nature to low acuity and low risk patients, provided in a unit setting, outside of the Medical Treatment Facility. The term was replaced by Integrated Operational Support Mental Health (IOS MH) with the publication of DAFMAN 10-211. (AFMAN 48-149)

Operational Clinical Services (OCS)—Clinical and clinical support services on ships and planes, in deployed settings, and in all other circumstances outside an MTF. (DoDI 6025.13)